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Name: اسماء محمد ابراهيم ابراهيم National ID: 27907220102987	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Everolimus therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
شهي
مطرة

MDT FORM

24/9/2023 G M

Name	اسماء ابراهيم	MRN	00111990
Age	44	ID	279.755.1.59
Menopausal status		Phone	010 7727780
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	RemDT

Clinical Data

BX: (R) IDC - GII

R

ER: 8/8 PR: 6/8 HER2: -ve Ki67: 50-60%

Sonomammography:

(R) breast multicentric locally advanced
BI-RADS, (L) → BI-RADS → path → no
malign

Mets Workup:

HA of (R) breast
5/2020 → NACT ACX4, taxol
then (R) MPM 1/2021 →

TNM

T2 N2 M1

Chemo

Recommendations:

PORT → TAM + Zoladex. Clo → bone pain,
bone scan 16/8/2023 → met. bone mets.

MDT OUTCOME & PLAN

Arimidex + CDK/4-6 inhibitor +
Zoladex.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



ImmunohistoChemistry report

Report Number: m3133

Female :	النوع	أسماء محمد إبراهيم	اسم المريض :
القاهرة :	العنوان	41 Y	السن :
13/06/2020 10:09 :	تاريخ استلام العينة		تاريخ اخذ العينة :
21/06/2020 10:33 :	تاريخ التقرير	مركز اورام دار السلام	طالب التحليل :
		مركز اورام دار السلام	غالبية :

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.

Sample recieved was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Sample recieved was treated by: - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: strong nuclear staining of > 60 % of tumour cells (core biopsy)

PR: moderate nuclear staining of 60 % of tumour cells (core biopsy)

HER2/neu protein: over expression : NO membrane staining of tumor cells (core biopsy)

Ki67 :positive nuclear staining of 50 - 60 % of tumour cells (core biopsy)

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 8 / 8)

PR: POSITIVE (SCORE 6 / 8)

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE

Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS 50 - 60 %

Pathologist:

د/تريزا ماهر

Lab Director:

د/تريزا ماهر



Pathology Report

Name: أسماء محمد إبراهيم

Age: 41 years.

Clinical data: Breast mass

Specimen type: Right breast MRM+ left WLE

Date: 19 /1/2021

Date(reported): /1/2021

Lab. Number: 123/2021

Hospital Number: 1892/2020 نفقة دولة

Gross Description:

Tow specimen:

A) Right MRM specimen with skin surface area 14x8cm including nipple and areole. serial cut sections revealed a retro areolar ill-defined rubbery grayish white area 3x2 cm, showing ectatic ducts oozing whitish material located 1P12

0.5 cm deep to skin of areola with least deep margin 2cm. Dissection of axillary tissue revealed 12 nodules up to 2x1.5cm.

B) Left breast WLE:

A piece of breast tissue 5x4x1.5cm showing ill-defined rubbery grayish white inducted area 1.5x1.3cm with least resection margin 0.6cm.

Macroscopic Examination:

histopathological examination of sections prepared from the right side reveals an infiltrating malignant epithelial growth made up of groups and sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. There is pathological partial response (PPR). Nipple, areola and margins are free. Four lymph nodes show metastatic deposits (4/12= N2).

histopathological examination of sections prepared from the left side reveals; diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells. There is no evidence of malignancy in sections examined.



امانة المراكز الطبية المتخصصة
مركز أورام دار السلام
إدارة المعمل
قسم الباثولوجى

Pathology Report

Name: اسماء محمد ابراهيم

Age: 40 years.

Clinical data: Right breast mass.

Specimen type: Tru cut biopsy.

Date: 2/6/2020

Lab. Number: 766

Hospital Number: /2020

Gross Description:

Multiple greyish white tissue cores, the longest was; 1.5 cm, totally submitted.

Microscopic Examination & Diagnosis:

Right breast mass, tru cut biopsy;

Invasive duct carcinoma GII, for IFT: (ER, PR, Her2 & Ki67).

Prof. Dr. Gamal Abdel-Aty

Dr. Marwa Mahmoud

Prof. Dr. Essam Ayad

Patient name: Asmaa Mohamed Ibrahim

Date: 7/11/2021

**LEFT BREAST MAMMOGRAPHY AND BILATERAL HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right MRM and left wire guided lumpectomy, with no left breast malignancy on pathology.

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- left breast tip of wire embedded in ?? clip (likely surgical), yet no related spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed left breast surgical interference with no related solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- Left breast clip with tip of wire with no related suspicious findings (BIRADS 3), for clinical assessment

Yours truly

Dr Engy Abdullah

MUHAMMED, ASMAA Patient ID: 124008533 DOB: 22-Jul-1979 Date: 05-Jan-2025

Patient Lastname: Muhammed

Patient Firstname: Asmaa

HIS Patient ID: 124008533

RIS Patient ID: 354628

Date of Birth: 22/07/1979

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

REPORT TEXT

99mTc MDP Bone Scan Report (5/1/2025)
Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP

Compared to previous study dated 16/05/2024 the current study revealed:

- " More or less stationary course of the metastatic osseous deposits at multiple skull bones, cervico-dorso-lumbar vertebrae, both gleno-humeral articulations, sternum and both sacroiliac joints.
- " Practical resolution of all other old metastatic lesions that had previously had a regressive course.
- " No newly developed osseous deposits.
- " Both Kidneys are visualized.

Conclusion: -

- " Rather stationary course of the old detected metastatic osseous lesions as described.

Much Obligated
Mahmoud Akram
Dr/Ahmed Zaher,MD

REPORT TEXT ENDE

Patient Name: Asmaa Mohamed Ibraheem **Date: 29/7/2025**

^{99m}Tc MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:
Referring physician: Prof.

Report:

- Follow up bone scan study report , compared to the last study dated 5/1/2025 , the current scan revealed :
- Rather stationary course of the previously detected active metastatic osseous lesions located at multiple skull bones , cervico-, dor-so-lumbar , vertebrae , both gleno-humeral articulation , the sternum , bilateral sacro-iliacs , .
- However , newly developed active osseous metastatic lesions are seen sacrum , bilateral ribs , multiple pelvic bones , (including Rt acetabulum , left femoral head) , for follow up .
- Both kidneys are normally seen.

Conclusion:

- Follow up bone scan , compared to the last study dated 5/1/2025 , the current scan revealed , progressive course of the disease as mentioned , for follow up .

Nuclear Medicine Unit

DR. Shorouk Mahdy, MD

DR/ Mohamed Emam

جمهورية مصر العربية
بطاقة تقييد الشخصية

اسماء
محمد ابراهيم ابراهيم
٢ ش ١٥٥ - ج نور الدين
المعادي - القاهرة

٢٧٩٠٧٢٢٠١٠٢٩٨٧

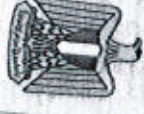
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حاصلة على بطاقات خدمة الخاصة

الاسم
محمد مصطفى حسن
البطاقة مصرية حتى ٢٠٣١/١/١٧

مستلة
مصرية

ADMS

Name : Asmaa Mohammed Ibrahim Ibrahim
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 18-09-2026 12:30
Medical No. : 318469
تعاقب مستشفى : Ins.Company :

١٢٥٨

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 47-year-old female with left breast carcinoma, status post mastectomy (January 2021), chemotherapy, radiotherapy, and current hormonal therapy. Known skeletal metastases. Presents with intermittent bone aches. Referred for follow-up skeletal evaluation compared to prior study dated February 16, 2026.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Serial comparison demonstrates mild overall regression in osteoblastic activity with reduced tracer uptake intensity across previously documented skeletal deposits, without complete resolution and without newly developed active deposits.
- Mildly regressive low-grade focal uptake is noted along the thoracolumbar spine and sacrum. Faint residual uptake is noted at the sternum and at the bilateral sacroiliac joints. A faint uptake shadow is noted over the right hip joint region (right acetabulum/femoral head).
- Mild symmetric uptake is noted at both shoulders, knees, and weight-bearing joints, consistent with degenerative changes.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Mild regression in osteoblastic activity across previously documented skeletal metastatic deposits compared to February 16, 2026.
- Minute residual uptake persists at the sacroiliac joints and sternum without complete resolution, and no newly developed active metastatic deposits.
- Faint uptake over the right hip joint is indeterminate; correlation with plain X-ray or CT is recommended.

Dr. Alaa Nagi

MD Nuclear Medicine



Medical No. : 10009

Patient's Name: Asmaa Mohammed Ibrahim Ibrahim

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE NECK,
CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST
INJECTION REVEALED:**

I- NECK:

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subcentimetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.

II- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

III- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Non visualized left kidney
- Right kidney is of normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No back pressure changes. few tiny stones are noted

Medical No. :

10009

Patient's Name:

Asmaa Mohammed Ibrahim Ibrahim

Date:

2026-09-17

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



- Normal C.T appearance of the spleen (apart from splenic calcifications), pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple vertebral, sternal both ilac bones lytic and sclerotic bony lesions

Opinion:

❖ Stationary course since the last study.

Dr.Asser Ahmed